Four-Day Week Pilots in Meridian, 2023–2025: Summaries and Findings

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Overview

This compendium summarises fourteen four-day week pilots run in Meridian between 2023 and 2025, covering 212 organisations and about 9,800 workers. Every participating organisation volunteered.

Each pilot used the "100-80-100" model: workers kept 100% of their pay for 80% of their usual hours, in exchange for a commitment to keep 100% of their output. Pilots ran for six months, and organisations then chose whether to continue.

The compendium is written for policymakers. It reports what the pilots found and, just as carefully, what they could not find.

1. Who took part

Seventy-one percent of participating organisations were in professional services, software, marketing or the non-profit sector. Only nine provided round-the-clock or shift-based services.

Sector	Organisations	Workers
Professional services	58	2,140
Software and digital	49	2,310
Marketing and media	21	640
Non-profit	23	910
Manufacturing	17	1,480
Retail and hospitality	35	1,620
Shift-based services	9	700
Total	**212**	**9,800**

Most organisations had fewer than fifty employees. Only six had more than five hundred.

2. Wellbeing

Across all pilots, self-reported burnout fell by an average of 32% and sick days by 21%. Revenue stayed broadly flat, but fewer than half of the firms tracked output with a consistent measure.

Workers reported better sleep, more time with family and less stress about household tasks. The improvements appeared within the first two months and held to the end of each pilot.

Improvements were smaller for managers, who often absorbed coordination work that had previously been spread across a five-day week.

3. What made pilots work

Firms with the biggest gains had halved their meetings and redesigned workflows before the trial began. Firms that simply removed a day without redesign saw smaller benefits and more overtime.

Common redesign steps included:

- cutting standing meetings to 25 minutes, or replacing them with written updates;
- "focus blocks" of two to three hours with no messages;
- clearing duplicated approval steps;
- agreeing in advance how to cover customers on the day off.

Firms that skipped redesign often saw workers squeeze five days of work into four, with longer days and weekend email. Their burnout reductions were about half the average.

4. The gap in the evidence

None of the pilots included an acute hospital, a public transport operator or a residential care provider. The authors flag this as the main gap in the evidence.

These sectors share a feature that most pilot firms did not: service must be provided at fixed times, whatever happens to productivity. A software team can finish a task faster; a bus still has to leave at 7:42.

The pilots therefore tell us a great deal about knowledge work, and much less about the sectors where a national standard would be hardest to apply.

5. Shift-based organisations

In the nine shift-based organisations, holding coverage constant took between 18% and 27% more staffed hours, met through new hires, overtime or agency workers.

The nine were three private clinics, two call centres, two security firms, a hotel group and a logistics depot. In each, managers redesigned rotas rather than hours, and the cost came from covering the hours that productivity gains could not replace.

Two of the nine reported that reduced sickness and turnover paid back part of that cost within the six months. The others did not measure it.

6. After the pilots

Ninety-two percent of organisations kept the four-day week after the pilot. Participants were not randomly selected, so the authors warn against reading this as a national estimate.

Organisations that volunteer for a four-day week pilot are likely to be the ones best placed to make it work. A firm that expects it to fail does not sign up.

6.1 Retention

Worker retention improved in most pilots: voluntary resignations fell from 6.1% to 4.3% on average over the trial period.

Recruitment also became easier. Several firms reported more applications per vacancy and fewer offers declined.

7. Recommendations

The authors recommend sector-specific trials in health, transport and care, with independent measurement of output and service quality, before any national standard is set.

In particular:

1. **Run trials where the gap is.** Fund at least one trial each in an acute hospital trust, a public transport operator and a group of residential care homes.
2. **Measure output independently.** Self-reported productivity is not enough. Use service-level data: waiting times, punctuality, care-quality inspections.
3. **Count both sides of the ledger.** Record the cost of extra staffed hours *and* savings from reduced sickness, agency use and turnover.
4. **Randomise where possible.** Assign wards, depots or homes to start dates at random, so that results can be compared fairly.
5. **Publish everything,** including trials that are stopped early.

8. Method and limitations

Results come from employee surveys at the start, midpoint and end of each pilot, alongside finance data reported by each organisation. Survey response fell from 74% to 58% by the final wave.

Falling response rates are a common problem in workplace trials. Workers who were unhappy with the pilot may have been less likely to answer the final survey, which would make results look better than they were.

Finance data was reported by the organisations themselves and was not audited. Output measures differed between firms; fewer than half used one consistent measure for the whole pilot.